

Subject: Food Package Assignment and Tailoring**Item: Food Package Tailoring**

PURPOSE:

To establish procedures for the proper assessment, assignment, and tailoring of authorized foods provided by the California Department of Public Health/Women, Infants and Children Division (CDPH/WIC).

POLICY:

- I. A Competent Professional Authority (CPA) must assess, assign, and tailor food packages to participants when appropriate to accommodate each participant's eligibility category, personal food or dietary preferences, and nutritional needs.

PROCEDURES:

- I. Food Package Assignment
 - A. CPA must assign a food package to each WIC participant and verify that the food package is appropriate in the WIC Web Information System Exchange (WIC WISE) prior to issuing benefits.
 - B. Medical documentation is required for issuance of medical formula and foods in Food Package III. Medical documentation is not required for other food packages.
- II. Food Package Tailoring – General
 - A. CPA has the option to tailor the participant's food package based on a complete nutritional assessment. Tailoring can include substituting (i.e., swapping) or removing a food item. The CPA must:
 1. Collaborate with the participant when tailoring the food package. The tailoring must be based on, but not limited to, the following:
 - a. Medical/nutritional needs.
 - b. Dietary preference due to cultural, religious, and/or lifestyle preference.
 - c. Household and food storage conditions (e.g., lack of refrigeration or homelessness).
 2. Make no assumptions about food preferences based on participant appearance, category, or cultural/ethnic/religious affiliation.
 3. Issue the full maximum monthly allowance (MMA) of food benefits to participants. Issuing less than the full amount is only appropriate under the following situations:
 - a. Medically or nutritionally warranted (e.g., removal of a food item due to an allergy/intolerance or another nutrition-related medical condition).
Document reason in Food Prescription screen.

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- i. Overweight and obesity are not medically or nutritionally warranted reasons to provide less than the maximum monthly allowance.
- b. Dietary or lifestyle preference (e.g., cannot use or prefers not to use the maximum amount; requests a combination of soy/dairy foods that is unavailable in an amount that would allow maximum issuance). Document reason in Food Prescription screen.
- c. Combo-feeding infants receiving formula. Refer to section VI. below for more information.

III. Food Package Tailoring – Dairy/Tofu

- A. The default non-infant food package in WIC WISE includes regular fluid cow's milk, yogurt, and cheese.
- B. The CPA can tailor non-infant food packages to include the following types/forms of dairy, tofu, and/or plant-based milk alternatives:
 - 1. Lactose-free milk
 - a. The CPA may issue lactose-free milk to participants who:
 - i. Are identified as lactose intolerant; or
 - ii. Request lactose-free milk based on preference, including but not limited to cultural, religious, and/or lifestyle.
 - 2. Evaporated milk
 - 3. Powdered dry milk
 - a. This option is only for participants 2 years of age or older.
 - 4. 2% Reduced fat milk and lactose-free 2% reduced fat milk
 - a. The CPA may issue 2% milk to children 12–23 months at risk for obesity, indicated by the assignment of Nutrition Risk 115 (High Weight for Length) in WIC WISE.
 - b. Children ages 12–23 months receiving Food Package III and at risk of obesity may also receive 2% milk as determined appropriate by their health care provider (HCP). Medical documentation is required. Refer to WPPM 390-10.
 - c. The family representative/caretaker may choose to receive the standard whole milk option for the 12-23 month old child. Issuance of 2% milk is not mandatory.
 - 5. Cheese
 - 6. Yogurt

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- a. Participants receiving non-infant food packages may choose to swap out a portion of their fluid cow's milk allowance for extra yogurt. The exception is the fully breastfeeding package. It already contains the maximum amount of yogurt in the default package.
- 7. Tofu and plant-based milk alternatives
 - a. The CPA may issue tofu and/or plant-based milk alternatives to participants 1 year of age or older who:
 - i. Are identified as lactose intolerant or allergic to cow's milk; or
 - ii. Request plant-based products based on preference, including but not limited to cultural, religious, and/or lifestyle.
 - b. All participants 1 year of age or older may swap any portion of their fluid cow's milk allowance for tofu and/or plant-based milk alternatives up to the MMA for milk.
- IV. Food Package Tailoring – Eggs
 - A. The CPA may issue tofu or legumes (i.e., peanut butter, canned mature beans, or dry beans, peas, and lentils) to participants who request these substitutes instead of their egg benefit(s).
- V. Food Package Tailoring – Fruits and Vegetables
 - A. Cash Value Benefit (CVB) for Children and Pregnant/Breastfeeding/Non-breastfeeding Participants
 - 1. The CPA must issue the full default dollar amount of the CVB, except when a participant 1 year of age or older is issued infant fruit and vegetable containers for a qualifying medical condition.
 - 2. Juice as Substitute for CVB
 - a. The CPA may issue juice to participants 1 year of age or older as a substitute for a specified amount of the CVB benefit.
 - B. Cash Value Benefit (CVB) for Infants
 - 1. The CPA may issue the CVB for fresh, canned, or frozen fruits and vegetables to infants on or after their 6-month birthdate. Participants can choose to receive the CVB in place of half or all of their infant fruit and vegetable containers.
 - 2. The CPA must perform a nutrition assessment to determine infant readiness before issuing CVB for infants.
 - a. For infants 5–7 months of age, the CPA must use the mid-certification appointment to fulfill the nutrition assessment requirement. The CPA must

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- assess infant readiness by asking the Nutrition screen questions on food intake frequency, current food intake, and food textures.
- b. For infants 8–11 months of age who did not complete the mid-certification appointment, the CPA must provide a modified nutrition assessment using the Nutrition screen questions on food intake frequency, current food intake, and food textures.
 3. The CPA must provide appropriate education to ensure that participants who will receive the CVB meet their nutritional needs in a safe and effective manner.
 - a. The CVB education must address the following topics:
 - i. Safe food preparation.
 - ii. Safe food storage.
 - iii. Feeding practices.
 - b. The CPA may fulfill the education requirement by reviewing one of the existing CDPH/WIC materials with the family representative/caretaker:
 - i. Baby Food for Me.
 - ii. Feed Me! 6–12 Months.
 4. The CPA must document the infant CVB nutrition education in WIC WISE.
 - a. For infants 5–7 months old who completed the mid-certification, document in the Care Plan Follow-Up Note per normal procedure as part of the mid-certification.
 - b. For Infants 8–11 months who did not complete a mid-certification and were assessed using the modified assessment, document “Infant CVB Assessment and Education” on the Nutrition Education (NE) Documentation screen.
 5. The CPA must add food prescriptions to appropriately reflect the CVB choice made by the family representative/caretaker.
- C. Infant Fruit and Vegetable Containers for Non-Infant Food Packages
1. The CPA may substitute infant fruit and vegetable containers for the CVB as determined appropriate by the HCP for participants 1 year of age or older.
 - a. Medical documentation is required. The medical documentation does not need to specify the amount of containers needed per day. Refer to WPPM 390-10.

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VI. Food Package Tailoring – Formula

- A. The CPA must assign the appropriate type of infant formula to meet the specific needs of the formula-feeding infant. The Primary Contract Brand Infant Formula is the formula of first choice.
- B. For a partially breastfeeding infant, the CPA must individually tailor the amount of infant formula based on the assessed needs of the breastfeeding infant. The CPA must ensure that the minimum amount of formula is provided to meet the infant's nutritional needs and breastfeeding participant's goals.
- C. The CPA must obtain medical documentation if the following is requested:
 - 1. Medical formula. Refer to WPPM 390-10 and 390-20.
 - 2. Increased formula when WIC infant food is restricted beyond six months of age. Refer to WPPM 390-10.
- D. Infants with Lactose Intolerance or Milk Allergy
 - 1. As appropriate, the CPA must issue either the authorized contract brand soy formula for infants with lactose intolerance or the prescribed medical formula to infants diagnosed with a milk allergy by an HCP. Issuance of medical formula requires medical documentation from the HCP. Refer to WPPM 390-10 and 390-20.

AUTHORITY:

[7 CFR §246.10](#)

[USDA Food Package Policy and Guidance March 2018](#)

CROSS REFERENCE:

WPPM 390-10 Provision of Therapeutic Formulas and WIC-Eligible Nutritionals

WPPM 390-20 Coordinating with Medi-Cal and Other Health Plans

RESOURCE:

[WIC Authorized Food List Shopping Guide](#)

Default Food Packages Staff Reference

WIC Food Swaps Staff Resource